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Challenges and opportunities for NGO-government collaboration in healthcare systems of developing countries

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ABSTRACT

Introduction: Governments worldwide increasingly emphasize collaboration with non-governmental organizations (NGOs) in healthcare. However, research on the challenges faced by NGOs, particularly in low- and middle-income countries, remains limited. This study aims to explore these challenges from the perspectives of healthcare managers and NGO leaders in Iran, a low-middle-income country.

Method: A qualitative study was conducted using 54 semi-structured interviews with managers from the governmental healthcare sector and active NGO leaders at the national level between 2023 and 2024. Data were analyzed using the framework analysis method in Atlas.T software.

Result: The study identified five key categories of challenges affecting NGO-government collaboration: political issues (e.g., policy instability and leadership changes), operational issues (e.g., inefficiencies in collaboration processes), cultural issues (e.g., mutual distrust between stakeholders), managerial issues (e.g., insufficient support and capacity-building), and communication issues (e.g., weak networks and lack of awareness about NGO roles).

Conclusion: This study offers a practical framework for addressing the identified challenges, particularly in developing countries. Specific recommendations include establishing consistent policies to ensure stability, simplifying collaboration procedures, and fostering mutual respect between NGOs and government agencies. These measures can enhance NGO effectiveness, ultimately improving healthcare service delivery and patient outcomes.

1. Introduction

Non-governmental organizations (NGOs) have emerged as crucial actors in addressing healthcare challenges worldwide. In many developing countries, public health systems are often overburdened, underfunded, and unable to meet the diverse needs of their populations (Besançon et al., 2022). NGOs play a pivotal role in filling these gaps by providing essential health services, advocating for underserved communities, and mobilizing resources to address pressing public health concerns (Sanadgol et al., 2021). Globally, NGOs have been instrumental in combating infectious diseases, improving maternal and child health, and supporting vulnerable populations in times of crisis (Palo et al., 2022). The significance of NGO-government collaboration has been recognized by international frameworks such as the Sustainable

Development Goals (SDGs), particularly Goal 3, which seeks to ensure healthy lives and promote well-being for all at all ages (WHO, 2019). Effective partnerships between governments and NGOs are essential for achieving universal health coverage and addressing systemic inequities in healthcare access. For instance, partnerships in countries like India have enabled NGOs to deliver tuberculosis care to remote populations, while collaborations in Kenya have facilitated the implementation of community health initiatives in underserved areas. However, these collaborations are often fraught with challenges, particularly in developing countries where political, operational, and cultural barriers impede the effectiveness of such partnerships (Joudyian et al., 2021).

In developing countries, political instability and frequent leadership changes can disrupt ongoing collaborations, creating uncertainty for NGOs and limiting their ability to plan and execute long-term projects

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(Adams, 2024). Bureaucratic inefficiencies, such as lengthy approval processes and unclear guidelines, further hinder the operational effectiveness of NGOs. Cultural misconceptions and mistrust between NGOs and government agencies are also common, exacerbating the difficulties in establishing mutually beneficial relationships (Bank, 2020). Addressing these challenges is critical not only for enhancing the effectiveness of NGOs but also for improving healthcare outcomes and resource utilization in these countries (Ogugua et al., 2024).

Iran, as a developing country with a complex healthcare system, exemplifies these challenges. The Ministry of Health and Medical Education (MoHME) in Iran recognizes the vital role of NGOs in addressing healthcare gaps, particularly in underserved and remote regions. Currently, over 600 healthcare-related NGOs in Iran actively contribute to activities such as providing financial support for patients, constructing healthcare infrastructure, and delivering specialized services. For example, NGOs supporting cancer treatment collectively serve thousands of patients annually and invest significant resources into improving healthcare accessibility (Aghababa S et al., 2015). Despite their contributions, Iranian NGOs face substantial barriers that undermine their potential impact. Political instability, inconsistent policies, bureaucratic hurdles, and cultural resistance are among the key challenges that limit effective NGO-government collaboration in the country (Saghravan and Moghimi, 2023).

While previous studies have explored the financial and operational contributions of Iranian NGOs (Saghravan and Moghimi, 2023; Hashemi et al., 2019; Rajabi et al., 2023), there is a lack of in-depth research examining the systemic barriers to their collaboration with government agencies. Understanding these barriers is critical for designing effective strategies to strengthen these partnerships, not only in Iran but also in other developing countries with similar healthcare challenges. This study aims to address this gap by analyzing the collaboration challenges faced by NGOs in Iran's healthcare system. By focusing on political, operational, cultural, managerial, and communication barriers, the study provides actionable insights and a practical framework that can inform policymaking and enhance NGO-government partnerships in developing countries.

2. Methodology

2.1. Study design and setting

This qualitative study was conducted between October 2023 to March 2024 to explore the challenges faced by non-governmental organizations (NGOs) in collaborating with Iran's healthcare system. The research used a descriptive-analytic approach, focusing on semi-structured interviews with healthcare managers and NGO leaders operating at the national level. The study targeted individuals with firsthand experience in NGO collaboration within the healthcare system.

2.2. Sampling and participants

Participants were selected using purposive and snowball sampling methods to ensure the inclusion of individuals with relevant expertise. Inclusion criteria for healthcare managers required at least one year of experience in the field of public collaboration at a national level. For NGO leaders, a minimum of one year of managerial experience and active involvement in healthcare activities were mandatory. A total of 54 participants were included in the study: 31 healthcare managers and 23 NGO leaders.

Potential limitations of the sampling method include the risk of selection bias due to the reliance on purposive and snowball sampling. These methods may exclude individuals with differing perspectives or those not actively engaged in prominent roles. To mitigate this limitation, the researchers ensured diversity in participant selection by including representatives from various NGOs and healthcare organizations across the country.

2.3. Data collection

Data were gathered through semi-structured interviews conducted in person, via telephone, and by email. Participants were contacted via phone or email to explain the study objectives and invite them to participate. For in-person interviews, appointments were scheduled based on participants' availability and convenience, with interviews conducted at their workplace, a neutral meeting space, or a location of their choice to ensure comfort and confidentiality. Phone and email interviews were also arranged at times suitable for the participants. Efforts were made to accommodate participants' schedules to facilitate meaningful engagement in the study. Of the 54 interviews, 48 were conducted in person, four by telephone, and two via email. In-person and phone interviews were recorded (with participants' consent), transcribed verbatim, and analyzed. For email interviews, participants provided written responses to the structured questions, which were directly coded and incorporated into the thematic analysis. Each interview lasted between 30 and 60 min and followed a standardized interview guide. The guide included open-ended questions developed based on previous Iranian studies in the field of healthcare NGO collaboration.

Participants were briefed on the research objectives, confidentiality of their responses, and the voluntary nature of participation before providing verbal consent. The interviews were recorded, transcribed verbatim, and reviewed for accuracy. Summaries were shared with participants at the end of each interview to confirm key points and clarify ambiguities. Data collection continued until data saturation was reached, meaning that no new themes or significant insights emerged from additional interviews. This approach ensured that the study captured a comprehensive range of perspectives on the challenges of NGO-government collaboration in healthcare.

To ensure the credibility of findings, we employed data triangulation by incorporating perspectives from multiple stakeholders, including NGO leaders and healthcare managers. This approach allowed for cross-validation of insights and reduced the likelihood of bias. However, we acknowledge that full triangulation, which would require the inclusion of policy and legal document analysis, was not conducted in this study. Future research should incorporate document reviews to further validate the findings and provide a more comprehensive assessment of NGO-government collaboration in healthcare.

Data were analyzed using the **framework analysis method**, a structured qualitative approach that ensures systematic organization and interpretation of findings. The analysis followed these five key steps:

Familiarization – Interview transcripts were thoroughly reviewed multiple times to achieve an in-depth understanding of the data and identify emerging patterns.

Developing a Thematic Framework – Recurring themes were identified based on participant responses, focusing on challenges in NGO-government collaboration within the healthcare system.

Indexing – A coding framework was developed, and data were coded line by line to identify patterns and insights. Codes were categorized into main and sub-themes, ensuring comprehensive data representation.

Charting – Coded data were systematically organized into matrices and tables, allowing for structured comparisons of themes across participants.

Mapping and Interpretation – Thematic relationships were synthesized to develop broader conceptual categories, leading to the identification of five primary challenges: **political, operational, cultural, managerial, and communication issues**.

To enhance the rigor of the study, we applied Guba and Lincoln's trustworthiness criteria:

Credibility: Data triangulation was conducted by involving both healthcare managers and NGO leaders to ensure a diverse range of perspectives. Additionally, member checking was performed by sharing summaries of findings with participants for validation.

Dependability: The coding process was conducted independently by two researchers, and discrepancies were resolved through discussion.

until consensus was reached. A transparent documentation process was maintained to enable future verification.

Confirmability: To reduce researcher bias, an audit trail was maintained, documenting analytical decisions, coding processes, and theme development.

Transferability: Detailed descriptions of the study setting, participant selection, and data collection processes were provided, allowing for potential application in similar contexts in other developing countries.

By incorporating these measures, the study ensures methodological rigor and enhances the reliability and validity of the findings.

2.4. Ethical considerations

This study was approved by the Medical Ethics Committee of the affiliated university (IR.MUMS.REC.1395.352). Ethical protocols included:

1. Obtaining informed consent from all participants.
2. Ensuring confidentiality and anonymity of responses.
3. Providing participants with complete information about the study's objectives and scope.

3. Results

The study included 54 participants, with 31 healthcare managers and 23 NGO leaders. Over 60 % of participants were male, and the majority (55 %) had 1–10 years of work experience. Most participants held at least a bachelor's degree, reflecting a relatively well-educated sample. These demographic characteristics are crucial for understanding the perspectives provided, as education levels and professional experience likely influenced participants' insights into collaboration challenges. For instance, less experienced managers may have encountered challenges differently than those with more extensive experience, while higher education levels may correlate with a broader understanding of policy and operational intricacies (Table 1).

Five major categories of challenges were identified, each comprising sub-concepts that provide deeper insights into the barriers faced by NGOs in collaborating with Iran's healthcare system. The obtained results are presented in Table 2.

3.1. Main concept number 1: political issues

Frequent changes in leadership within the healthcare system and inconsistent government policies emerged as significant barriers. Participants highlighted that leadership changes often disrupt NGO-government collaborations, creating uncertainty for ongoing projects. For example, participants reported that changes in university leadership resulted in the cancellation of previously approved initiatives. "The other problem is that changing the manager regularly [at the University of Medical Sciences] makes collaborations less and that's a lot" (p 21). "a manager welcomes a donor, but tomorrow, another university manager

Table 1
Demographic characteristics of Interviewees.

Demographic date		managers of healthcare system	Manager of NGO's	Total Percent
Gender	Female	6	5	31.37
	male	11	10	68.62
Education	Bachelor	6	5	31.37
	Ms	11	6	53.12
	PhD	3	1	12.5
Year of Experience	1–10 years	3	7	31.25
	11–20 years	12	8	62.5
	21–30 years	2	–	6.25
	years			

Table 2

Main and subsidiary concepts of collaboration challenges of non-governmental organizations in healthcare from the viewpoint of healthcare managers and managers of non-governmental organizations.

Main Concepts	Subsidiary Concepts	Description
Political issues	Effects of politics on NGO s	Political influence on NGO operations and restrictions due to policy changes.
	Role of government	Extent to which government authorities control, support, or limit NGO activities.
	Role of laws	Legal frameworks governing NGO operations and how they impact collaboration.
Performance issues	Motivational Factors	Factors that influence NGO engagement, sustainability, and commitment.
	Coordination Challenges	Difficulties in aligning NGOs with governmental processes and services.
	Organizational Barriers	Internal challenges within NGOs affecting structure and operational efficiency.
	Planning Issues	Strategic and procedural deficiencies affecting NGO-government partnerships.
Cultural issues	Financial Constraints	Limited funding sources and resource allocation issues that hinder NGO activities.
	Cultural Infrastructures	Existing cultural frameworks and how they affect NGO integration and acceptance.
	Reforming Attitudes	Need for attitude shifts among stakeholders to foster a more collaborative environment.
Managing issues	Public Perceptions	Public awareness and misconceptions affecting NGO credibility and engagement.
	Role of Governmental Managers	Government managers' leadership role in fostering or hindering NGO partnerships.
	Role of NGO Managers	Effectiveness of NGO managers in navigating bureaucratic and administrative hurdles.
Communication issues	Managerial Capacity	Capacity gaps in NGO leadership that affect strategic decision-making.
	communication Gaps and Barriers	Challenges in information-sharing, lack of structured communication channels, and misunderstandings.

fires the same donor. This happens a lot and makes the donor confused and uncertain" (P 7,P 20,P 17). Government should remain the sole decision maker, not the operator, and people should take the lead of their own affairs, For instance, "it can be said that Different political factions give different meaning to. Ups and downs of each government are different" (P 9,P 1, P6, P8).

The top-down view of the public sector towards NGOs has led to grammatical behaviors of public employees with NGOs, which has led to the discontent of NGOs and their diminished engagement with government areas "Sometimes when we [NGOs] had trouble [requesting the University of Medical Sciences], but we were worried about their collusion" (p28).

The lack of public sector Supervision of NGOs in the health sector causes some NGOs to fail to comply with the guidelines "Government must monitoring of semen performance and their activities " (p16).

During the interviews, some participants stated that in addition to political abuses, the government is also abusing NGO s" ... They want us to support the political work of government agencies, but we do not engage ourselves. (p21).

These findings suggest the need for policy continuity and leadership stability to foster trust and long-term collaboration between NGOs and government agencies.

3.2. Main concept number 2: operational issues

Participants described how bureaucratic inefficiencies, such as lengthy approval processes and inconsistent communication, hindered the operational effectiveness of NGOs. One participant noted that securing resources often required navigating complex and repetitive procedures. "... in Iran we expect NGO s to be different from other countries. It is very important that people say their health needs and their suggestions and where they can be present should be specified ..." (p30).

The NGO s Board of Trustee includes a number of people who sometimes do not share a common view of how to pursue a path to accomplish activities and achieve the goals. "Unfortunately, teamwork is difficult ... One of the problems in NGO s is the difference in taste causing them not to perform or to move elsewhere" (p27).

"An adroit person receives services from several sources, but another person who is unable in this regard will be deprived of services, and parallel processes intensify this problem" (P 3, P5, P11, P12). "for receiving facilities from the governmental sector, there is a long process ... if I [manager of NGO s] want to get help from someone in the field of healthcare, no one is responsible and I should refer and have a long talk from early morning each time" (P 12, P11, P 1, P18). "Hospitals have no specific pattern in relation to NGO s and are provided with insufficient information about the NGO s." (P 11, P12, P17).

"All the affairs should be carried out with a certain coordination. Awareness should be increased, especially awareness of Hospitals from their interactions with donors and NGO s" (P 4, P11, P17).

Streamlining administrative processes and establishing clear operational guidelines could enhance NGO productivity and reduce frustration among stakeholders.

3.3. Main concept number 3: cultural issues

Lack of appropriate infra-structures with the aim of institutionalizing non-governmental organizations is another problem leading to weak performance of NGO s. "Collaboration should be cultural and educational, and healthcare NGO s should initiate with the help of municipality." (P 6, P18).

Cultural barriers were reported as a significant challenge, with participants citing resistance to collaboration from public sector entities. Misconceptions about the motives of NGOs, such as the belief that they compete with government agencies, further complicated relationships.

"our main challenge [healthcare NGO s] is that they think we work internationally and they do not cooperate with us" (P 1, P4, P11). So this leads to positioning of some of the governmental managers against NGO s and reduction of their collaboration." Health Centers don't cooperate ... practically the public sector they just looking to see what we're going to do "(p14) It is recommended that this problem can be solved via counseling and holding meetings and memorandums of understanding. Some of the governmental healthcare managers believe that NGO s, who proceed to work in healthcare system with the aim of founding their organization, (for example for making money)." Unfortunately, there is a bad outlook in our society ... there is no place for our people to say, for example, that I want to do something good" (p29) Another challenge is lack of people's awareness from charity organizations which causes some challenges for these organizations in Iran. "People should directly enter the field of charity, however there is a lack of awareness from donors and performances of social deputy ..." (P 3, P 9, P11). "In Iran, what is referred to as non-governmental organization is not inferred similarly by all the people" (p25).

Initiatives to build mutual trust, foster a culture of teamwork, and improve public awareness of NGOs' roles could mitigate these challenges.

3.4. Main concept number 4: management issues

Both healthcare managers and NGO leaders identified weak support from government managers as a critical issue. Participants emphasized the importance of empowering NGO leaders through training programs and stronger managerial support. One participant highlighted the need for transparent and competent leadership to build trust and sustain collaborations., "the manager should believe in NGO s ... a reason for detachment of NGO s is lack of governmental supports" (P 2, P5, P7, P11, P18). Managers of non-governmental organizations need governmental supports for progress and fulfillment of their goals. "The manager has to believe that the benefactors can help ... extend their hand to them." (p18).

Lack of scientific management can be considered as a main cause of inefficiency in charity organizations "manager should be trustful, known, and honest; so that we do not think of deceiving him/her [healthcare NGO s and the donor] in order to take money and spend it in another field" (P 1, P5, P6, P9).

In addition to using NGO s to solve problems, managers also need to increase their impact on society " ... the most important thing a manager can do is empower NGO s So they can be influential at the community level" (p24).

Some interviewees have emphasized on the presence of clinical specialists (benefactor) in board of directors of NGO s, the effects of manager's reputation on continuity of the activities, and Managers incapacity of some NGO s managers as other challenges in this area. As an example, "due to the presence of some of the board members [NGO s] in some places, our credit is risen [healthcare NGO s] and we can achieve further credits [from governmental sector]" (P 11, P12, P14, P16). In order to continue the partnerships of NGO s or Benefactors with public health authorities, there should be managers who are well-known and trusted, not managers who do not have these capabilities." "The person who is hired as a manager must be trustworthy ... must be known ... the most important characteristic of a manager ... must be honest." (p27).

Capacity-building initiatives, leadership development programs, and transparent managerial practices are essential for addressing these challenges.

3.5. Main concept number 5: communication issues

Weak communication channels between NGOs and government agencies were frequently mentioned. Participants reported that a lack of structured communication pathways led to misunderstandings and reduced collaboration effectiveness. One NGO leader noted that public healthcare managers were often unaware of the scope of NGO activities, resulting in missed opportunities for collaboration. "There is no regular communication with NGO s ... we should look after them, while they should look after us, and it is not good ... the main problem is informing the hospitals about us" (P 12, P 11, P 18).

Furthermore, another challenges, is unspecific number of NGO s and their fields of activities in healthcare system for creation of appropriate communication. As we know, lack of active and appropriate network communication between non-governmental organizations causes a lot of challenges in their collaboration with healthcare governmental sector. It is suggested to make some flowcharts for communication paths of healthcare NGO s in different meetings in order to overcome these challenges more optimally and increase their collaboration.

Lack of early communication between the public health authorities and active semen in this area was another major challenge " ... communication should be made with this semen to provide guidelines and update their information." (p14).

Another challenge the researcher has encountered in the interview process is the inadequate knowledge of public sector staff about health NGO, s, as well as the lack of job recognition among health NGO,s themselves" If we can figure out how to use health NGO s, we can be successful ... we must have at least a few meetings with them annually

... to make them more known.” (p30).

Establishing formal communication protocols and regular joint meetings could improve alignment and foster productive partnerships.

4. Discussion

The findings of this study highlight five major barriers that hinder effective collaboration between NGOs and government agencies in Iran's healthcare system: political, operational, cultural, managerial, and communication challenges. These barriers align with findings from previous studies in other developing countries (e.g., India, Kenya, and Ghana), demonstrating that such challenges are systemic and not unique to Iran.

1. **Political Issues:** Frequent leadership changes and inconsistent policies disrupt long-term NGO-government collaborations, leading to uncertainty in project implementation. Similar political instability has been observed in Sudan, where fluctuating healthcare priorities affect NGO contributions (Charani et al., 2019).
2. **Operational Barriers:** Bureaucratic inefficiencies, such as lengthy approval processes and lack of standardized collaboration frameworks, slow down NGO activities. A comparable issue exists in India, where complex administrative structures create barriers to NGO involvement (Kumar, 2023).
3. **Cultural Challenges:** Mistrust and misconceptions about NGO roles create resistance from public healthcare officials. Myanmar faced similar challenges, which were mitigated through stakeholder dialogues and collaborative training sessions (Than et al., 2024).
4. **Managerial Limitations:** Weak support from government agencies and insufficient leadership capacity within NGOs limit their impact. Capacity-building programs in Ghana have demonstrated the importance of leadership training in overcoming such barriers (Maweale, 2023).
5. **Communication Gaps:** A lack of structured networks and information-sharing mechanisms reduces collaboration effectiveness. Rwanda addressed this by implementing centralized communication platforms, improving NGO-government interactions (Twizeyimana et al., 2018).

These findings reinforce the need for policy reforms, improved operational efficiency, trust-building initiatives, leadership development, and enhanced communication strategies to strengthen NGO-government partnerships in healthcare.

Another strength of this study is its ability to provide actionable and policy-relevant findings. By identifying key challenges and categorizing them into political, operational, cultural, managerial, and communication issues, the study offers a structured framework that can be utilized by both policymakers and NGO leaders to enhance collaboration. The practical recommendations, such as streamlining bureaucratic processes, fostering trust, and improving communication networks, are directly informed by real-world stakeholder experiences, making them applicable beyond Iran's healthcare system. The use of multiple interview methods (in-person, phone, and email) further enhances the depth of the collected data, ensuring inclusivity and capturing a wide range of viewpoints.

4.1. Contextualization of results in relation to existing literature

The findings of this study align with and expand upon existing research on NGO-government collaboration in healthcare systems, particularly in low- and middle-income countries (LMICs). This section situates the results within the broader academic discourse, demonstrating how they reinforce, challenge, or extend previous work.

1. **Political Barriers:** Instability and Policy Inconsistencies

The study highlights that frequent leadership changes and policy fluctuations disrupt NGO-government collaboration. This finding is consistent with Adams (2024), who identified governance instability as a key determinant of ineffective health partnerships in underdeveloped nations. Similarly, Charani et al. (2019) observed that policy inconsistencies in Sudan led to discontinuities in tuberculosis care programs managed by NGOs. The results reinforce the argument that institutionalizing NGO participation in national health policies, as seen in Kenya's National NGO Coordination Act (Hussein et al., 2021), could mitigate these issues.

2. **Operational Inefficiencies:** Bureaucratic Hurdles and Administrative Barriers

Bureaucratic inefficiencies, such as lengthy approval processes and lack of coordination, were identified as major barriers. Kumar (2023) found similar challenges in India, where excessive regulatory requirements delay NGO-led rural healthcare interventions. A comparative case from Rwanda (Twizeyimana et al., 2018) demonstrates that digitized administrative systems significantly improve NGO engagement by streamlining approvals. The current study's findings support the recommendation that e-governance mechanisms could enhance operational efficiency in Iran's healthcare system.

3. **Cultural Barriers:** Mistrust and Lack of Public Awareness

The study's findings reveal mutual distrust between NGOs and public sector officials, as well as a lack of public awareness regarding NGO roles. These findings align with Than et al. (2024), who observed similar resistance in Myanmar, where NGOs were initially perceived as competitors rather than collaborators. Myanmar's stakeholder engagement initiatives, which included collaborative training and joint projects, successfully altered public sector attitudes. The results suggest that Iran could benefit from structured stakeholder dialogues and trust-building programs, reinforcing earlier research by Sanadgol et al. (2022) on NGO integration into national health systems.

4. **Managerial Weaknesses:** Lack of Support and Capacity-Building Gaps

Weak managerial support and inadequate training for NGO leaders were identified as critical barriers. This is consistent with Maweale (2023), who found that in Ghana, leadership training for NGO managers significantly improved their ability to navigate bureaucratic structures and secure funding. The current study supports the argument that structured leadership development programs can strengthen NGO sustainability and effectiveness, a finding also echoed in Agyepong and Adjei (2008) in their assessment of Ghana's health insurance framework.

5. **Communication Gaps:** Weak Networks and Information Asymmetry

The study underscores poor communication between NGOs and government agencies, leading to inefficiencies and missed collaboration opportunities. This aligns with Twizeyimana et al. (2018), who identified the lack of formalized communication pathways as a barrier in Rwanda. Their implementation of centralized communication platforms facilitated better coordination and policy alignment. The findings indicate that Iran could benefit from a similar structured communication network, ensuring regular dialogue between NGOs and healthcare authorities.

4.2. Expanded contribution to existing literature and policy implications

This study not only confirms previous research findings but also extends the literature by providing a structured framework for analyzing NGO-government collaboration barriers in Iran. Unlike previous studies,

which often focus on a single aspect (e.g., financial or operational constraints), this research integrates political, operational, cultural, managerial, and communication barriers, offering a comprehensive model for evaluating collaboration challenges. By categorizing these barriers into a structured framework, this study enables policymakers and researchers to examine the interconnected nature of these challenges rather than treating them as isolated issues. This holistic approach provides a more nuanced understanding of the complex dynamics shaping NGO-government relationships in healthcare.

Additionally, this study contributes methodological advancements to the field of NGO collaboration research. By employing framework analysis and incorporating Guba and Lincoln's trustworthiness criteria, the study ensures a systematic and replicable approach to qualitative data analysis. While prior studies have often relied on descriptive analyses of NGO-government interactions, this research provides a structured coding system that can be adapted for future studies in similar contexts. The iterative analysis process strengthens the reliability of the findings, ensuring that the identified themes are not only representative of participants' perspectives but also aligned with broader trends observed in other low- and middle-income countries (LMICs).

The results suggest that Iran, and other LMICs facing similar constraints, could benefit from the adoption of key policy measures, including:

Institutionalized NGO participation frameworks (e.g., Kenya's policy model): Establishing formal policies that define NGO roles within healthcare governance ensures stability and prevents political disruptions that hinder long-term collaboration. By creating legal protections for NGO involvement, countries can foster an environment where NGOs are integrated into public health strategies rather than treated as external actors.

Digitization of bureaucratic procedures to reduce inefficiencies: Implementing e-governance solutions can help streamline administrative processes, reducing delays in NGO registration, funding approvals, and reporting. Digital platforms can also improve transparency and accountability in NGO-government interactions, mitigating the risk of bureaucratic inefficiencies that often hinder collaboration.

Trust-building mechanisms through stakeholder engagement: Establishing regular forums for dialogue between NGOs and government officials can help address misconceptions and build mutual respect. Stakeholder engagement strategies, including joint training programs, co-governance initiatives, and cross-sector working groups, can enhance alignment between NGO and governmental priorities, leading to more effective partnerships.

Capacity-building programs for NGO managers: Strengthening leadership within NGOs through structured training programs can help managers navigate complex bureaucratic environments and secure sustainable funding. These programs should focus on policy literacy, financial management, strategic planning, and negotiation skills, equipping NGO leaders with the tools needed to engage effectively with government institutions.

Centralized communication networks to improve information flow: Establishing dedicated communication platforms can facilitate knowledge-sharing between NGOs and government agencies. A centralized digital database of NGO activities, funding sources, and service coverage can help governments coordinate efforts more effectively, reducing duplication of services and ensuring that resources are allocated where they are most needed.

By situating these findings within the broader academic discourse, this study reinforces the global relevance of NGO-government collaboration challenges while offering practical, policy-oriented solutions. The structured framework presented in this study provides a replicable model for future research, allowing scholars to analyze NGO-government dynamics in different political and economic contexts. Furthermore, the policy recommendations outlined in this study can serve as actionable guidelines for governments and NGOs seeking to strengthen their partnerships.

Future research should focus on evaluating the real-world impact of policy interventions in countries that have successfully mitigated these barriers. Comparative studies between nations with high-functioning NGO-government partnerships (e.g., Kenya, Rwanda, and Ghana) and those where collaboration remains weak (e.g., Iran, Sudan, and Myanmar) can provide deeper insights into the conditions necessary for effective integration. Additionally, research examining longitudinal data on NGO-government collaborations can help assess whether the proposed solutions lead to sustained improvements in healthcare accessibility, quality, and efficiency.

Ultimately, by adopting a structured, evidence-based approach to NGO-government collaboration, countries can optimize healthcare delivery, improve health outcomes for underserved populations, and ensure that NGOs serve as integral partners in achieving national and global health goals, including Universal Health Coverage (UHC) and the Sustainable Development Goals (SDGs).

4.3. Implications for developing countries

4.3.1. Political stability and institutionalization

The study demonstrates how political instability and frequent leadership changes disrupt NGO-government collaborations by undermining trust and continuity. This issue is widespread in developing countries, where governance instability often results in fluctuating healthcare priorities. For instance, in Sudan, frequent policy shifts have led to delays in NGO-implemented tuberculosis care programs (Charani et al., 2019). To address this, developing countries can adopt regulatory frameworks that institutionalize NGO roles within healthcare systems, such as Kenya's National NGO Coordination Act, which ensures NGOs remain integral to health policy implementation despite political changes (Hussein et al., 2021). Adapting such frameworks can foster policy stability, ensuring NGOs can plan and execute long-term projects effectively.

4.3.2. Streamlining operational processes

Operational inefficiencies, such as lengthy approval processes and unclear guidelines, were significant barriers identified in this study. These inefficiencies are prevalent in many developing countries, as seen in India, where complex bureaucratic procedures often delay NGO activities in rural healthcare (Kumar, 2023). Rwanda's adoption of digital platforms to streamline NGO registration and project tracking offers a model for overcoming such barriers (Twizeyimana et al., 2018). By digitizing administrative processes and providing NGOs with clear operational guidelines, governments can reduce delays, enhance transparency, and optimize resource allocation. Countries like Iran could benefit from creating centralized e-governance systems that facilitate real-time communication and approvals between NGOs and governmental bodies.

4.3.3. Building trust and addressing cultural barriers

Cultural misconceptions and mistrust between NGOs and government agencies were recurring themes in this study. Similar challenges have been reported in Myanmar, where NGOs were initially perceived as rivals to public health authorities (Than et al., 2024). To overcome this, Myanmar introduced trust-building initiatives, including collaborative training sessions and regular stakeholder dialogues. These measures successfully enhanced partnerships between NGOs and public health authorities, enabling smoother implementation of healthcare projects (Than et al., 2024). In Iran and other developing countries, fostering trust through joint workshops, public awareness campaigns, and formal agreements could redefine NGO-government relationships and enhance collaboration.

4.3.4. Strengthening leadership and management capacity

Weak leadership and insufficient managerial support for NGOs emerged as critical issues in this study. Such challenges are also evident

in Ghana, where NGO leaders faced difficulties in engaging with government stakeholders due to a lack of structured leadership training programs (Mawele, 2023). Ghana's introduction of mentorship initiatives and capacity-building workshops significantly improved NGO managers' effectiveness and their ability to secure governmental support (Abadzi). Developing countries can adapt these strategies by investing in leadership development programs that enhance the managerial skills of NGO leaders. Transparent performance evaluation systems and mentorship from experienced leaders can further strengthen the capacity of NGOs to navigate complex healthcare systems.

4.3.5. Enhancing communication networks

Communication gaps, including weak networks and unstructured interactions between NGOs and government agencies, often lead to misunderstandings and missed opportunities for collaboration. This study's findings align with those from Rwanda, where inadequate communication between stakeholders hindered the efficient implementation of healthcare initiatives (Twizeyimana et al., 2018). Rwanda addressed this challenge by establishing centralized communication platforms that facilitated regular updates, transparent information sharing, and joint planning sessions (Twizeyimana et al., 2018). Developing countries can adapt similar platforms to improve communication and coordination between NGOs and government agencies. Additionally, regular joint forums and annual stakeholder meetings could foster alignment on healthcare priorities and enable collaborative problem-solving.

4.4. Universality of recommendations and broader adaptation

The findings of this study are not only relevant to Iran but also reflect broader systemic challenges faced by NGOs in healthcare systems across many low- and middle-income countries (LMICs). The barriers identified—political instability, bureaucratic inefficiencies, cultural resistance, weak managerial capacity, and poor communication—are well-documented in various global contexts. This suggests that the recommendations proposed in this study can be adapted to diverse healthcare systems, particularly in countries where NGOs play a crucial role in delivering healthcare services and bridging gaps left by underfunded public health infrastructures.

One of the key recommendations, institutionalizing NGO roles in healthcare governance, has already been successfully implemented in several countries. For example, Kenya's National NGO Coordination Act ensures that NGOs remain integrated into national healthcare planning, regardless of political changes. By formalizing NGO-government partnerships, such policies create a stable environment for long-term collaboration. Similar regulatory frameworks could be adapted in Iran and other developing nations to prevent the disruption of NGO activities due to shifts in political leadership or inconsistent government policies.

Another crucial recommendation, streamlining bureaucratic processes, has proven effective in countries like Rwanda, where digital platforms have been introduced to manage NGO registration and project approvals efficiently. This has significantly reduced administrative delays and enhanced transparency in government-NGO interactions. By adopting digital solutions for NGO licensing, funding applications, and communication with public health authorities, countries facing bureaucratic hurdles—such as India, Sudan, and Iran—can improve operational efficiency and ensure smoother collaboration between NGOs and government agencies.

Building trust between NGOs and government entities is another universally applicable recommendation. In many LMICs, NGOs are often viewed with suspicion, either as competitors to government services or as organizations with external political or financial interests. Myanmar has addressed this issue by introducing structured stakeholder engagement programs, including collaborative training sessions, joint task forces, and public awareness campaigns to improve perceptions of NGOs. By fostering an environment of cooperation, governments in Iran

and other LMICs can create more inclusive healthcare strategies that leverage NGOs as essential partners rather than parallel service providers.

The recommendation to strengthen managerial capacity within NGOs has been effectively implemented in Ghana, where leadership development programs have significantly enhanced NGO effectiveness. Through structured mentorship initiatives and government-supported capacity-building workshops, Ghanaian NGO leaders have become more skilled at navigating policy landscapes and securing sustainable funding. A similar approach could be highly beneficial in Iran, where many NGOs struggle due to limited administrative expertise and weak institutional support. Providing structured training for NGO leaders on financial management, healthcare policy, and regulatory compliance can improve their ability to collaborate effectively with government institutions.

Improving communication channels between NGOs and government agencies is another critical area where best practices from other countries can be adapted. In Rwanda, the establishment of centralized communication platforms has significantly improved the efficiency of NGO-government interactions. By creating designated liaison offices or digital platforms for NGO coordination, developing countries can minimize misunderstandings, reduce redundancy in healthcare services, and foster a more cohesive approach to public health initiatives. Countries such as Iran, where fragmented communication networks hinder collaboration, could benefit greatly from the adoption of similar mechanisms.

The universality of these recommendations highlights that while each country has unique political, cultural, and operational contexts, the fundamental challenges facing NGO-government collaboration are widely shared. The success stories from countries such as Kenya, Rwanda, Myanmar, and Ghana demonstrate that targeted interventions—such as legal frameworks, digital governance, trust-building programs, leadership development, and structured communication networks—can be effectively adapted to different settings.

Future research should focus on evaluating the effectiveness of these strategies in different healthcare systems and exploring how contextual factors influence the success of NGO-government collaboration. Comparative studies between countries that have successfully institutionalized NGO roles and those still facing challenges could provide further insights into best practices. Additionally, pilot projects implementing these recommendations in specific regions could serve as models for broader national adoption.

By adapting these proven strategies, developing countries can enhance the sustainability and impact of NGOs in healthcare, improve healthcare accessibility, and create more resilient health systems that effectively address the needs of vulnerable populations.

4.5. Future research and practical implications

This study contributes to the growing body of knowledge on NGO-government collaborations in healthcare, offering a framework for addressing common barriers. Future research should evaluate the long-term impact of the proposed strategies, particularly in contexts where they have been successfully implemented. Additionally, exploring the perspectives of community members and healthcare beneficiaries could provide deeper insights into the effectiveness of NGO-government partnerships. Policymakers in developing countries can leverage these findings to design tailored interventions that enhance collaboration, optimize resource utilization, and improve healthcare outcomes.

4.6. Limitations of the study

While this study provides valuable insights into the challenges of NGO-government collaboration in Iran's healthcare system, several limitations should be acknowledged:

1. Sampling Limitations and Potential Bias

The study employed purposive and snowball sampling, which, while effective for identifying knowledgeable participants, may have introduced selection bias. Participants were primarily experienced NGO leaders and healthcare managers, potentially excluding perspectives from smaller, less-established NGOs or mid-level government officials who might have different insights.

Future research should consider broader sampling techniques, such as randomized or stratified sampling, to capture a more diverse range of viewpoints.

2. Self-Reported Data and Social Desirability Bias

Since data were collected through semi-structured interviews, responses may have been influenced by social desirability bias, where participants provided answers they believed were more socially or politically acceptable rather than completely candid perspectives.

To mitigate this, future studies could incorporate triangulation methods, such as observational studies, policy document reviews, or quantitative surveys, to cross-validate findings.

3. Mode of Interviews and Data Collection Constraints

While most interviews were conducted in person, some were conducted via phone and email, which may have limited the depth of responses compared to face-to-face interactions. Email interviews, in particular, lacked the ability to capture non-verbal cues and spontaneous elaborations that could have enriched the data.

Future research could focus on conducting only in-person or video interviews, ensuring a more interactive and detailed discussion with participants.

4. Limited Generalizability Beyond Iran

Although the study provides valuable insights applicable to other developing countries, the findings are context-specific to Iran's healthcare system and its unique political, cultural, and administrative landscape.

While similar challenges exist in other LMICs, caution must be exercised when applying these findings to different regions, as healthcare governance structures vary significantly. Future research should conduct cross-country comparisons to enhance generalizability.

5. Lack of Longitudinal Data

This study provides a snapshot of current NGO-government collaboration challenges but does not capture how these challenges evolve over time.

A longitudinal study would provide deeper insights into the impact of policy changes, leadership shifts, and NGO-government initiatives over time.

6. Lack of Quantitative Validation

The study primarily relies on qualitative data, which, while rich in detail, lacks quantitative validation that could strengthen findings.

Future studies could incorporate mixed-method approaches, including survey-based quantitative analysis, to measure the extent of each challenge and its impact on healthcare outcomes.

One limitation of this study is the lack of a documentary analysis of legal and policy frameworks governing NGO-government collaboration in healthcare. While the study relies on firsthand accounts from key stakeholders, future research should include a comprehensive review of policy documents, governmental regulations, and legislative texts to strengthen data triangulation and provide a broader contextual

understanding of collaboration barriers.

Despite these limitations, this study provides a structured and in-depth exploration of the barriers to NGO-government collaboration in healthcare. By acknowledging these constraints, future research can build on these findings using broader sampling methods, mixed-method approaches, and longitudinal analysis to further enhance the applicability and robustness of the results.

4.7. Conclusion

This study provides a comprehensive analysis of the challenges affecting NGO-government collaboration in Iran's healthcare system, identifying five major barriers: political, operational, cultural, managerial, and communication challenges. These findings contribute to the existing body of knowledge by offering a structured framework for understanding and addressing systemic obstacles in developing countries' healthcare systems. The study highlights political instability, bureaucratic inefficiencies, mistrust, insufficient managerial support, and weak communication networks as major barriers to effective collaboration. These challenges are not unique to Iran but are also prevalent in other low- and middle-income countries. Our findings reinforce previous research conducted in India, Kenya, Ghana, Rwanda, and Myanmar, emphasizing the need for policy stability, streamlined administrative processes, and trust-building initiatives to enhance NGO engagement in healthcare service delivery.

A key strength of this study is its methodological rigor, employing framework analysis to ensure a systematic, transparent, and structured examination of qualitative data. By engaging both NGO leaders and healthcare managers, the study captures multiple stakeholder perspectives, increasing the credibility of the findings. Additionally, by applying Guba and Lincoln's trustworthiness criteria—credibility, dependability, confirmability, and transferability—the study strengthens its reliability and practical applicability. This reinforces the relevance of the findings not only for Iran but also for other developing countries facing similar healthcare governance challenges.

To enhance NGO-government collaboration in healthcare, policy-makers and healthcare administrators should consider several recommendations. Institutionalizing NGO roles in healthcare governance through formalized policies can ensure stable partnerships, regardless of leadership changes. Adopting regulatory models such as Kenya's National NGO Coordination Act could help integrate NGOs into national health policy structures. Bureaucratic inefficiencies can be mitigated through the digitization of administrative processes, simplifying NGO registration, funding applications, and project approvals, as successfully demonstrated in Rwanda's e-governance initiatives. Building trust between NGOs and government agencies is another critical step. Implementing collaborative training programs, stakeholder meetings, and public awareness campaigns can reduce misconceptions and mistrust. Myanmar has successfully used joint training sessions to strengthen NGO-government relations, a model that could be adapted to the Iranian healthcare context. Strengthening leadership and managerial capacity within NGOs is also essential, as seen in Ghana, where capacity-building programs have improved NGO leaders' ability to navigate policy and funding challenges. Additionally, establishing formal communication networks between NGOs and government agencies can enhance coordination and effectiveness. Creating annual multi-stakeholder forums could foster policy alignment and improve long-term collaboration.

While this study provides valuable insights, it has certain limitations, including sampling constraints, reliance on self-reported data, and the absence of quantitative validation. Future research should expand the sample to include a broader range of NGOs, mid-level government officials, and healthcare beneficiaries. Longitudinal studies are needed to assess how NGO-government collaboration evolves over time, particularly in response to policy changes and leadership shifts. Additionally, mixed-method approaches incorporating both qualitative and quantitative analyses could help quantify the impact of identified challenges

and assess the effectiveness of policy interventions.

Overall, this study underscores the critical role of NGOs in bridging healthcare gaps in developing countries and the urgent need for policy interventions to improve their collaboration with governments. By addressing political, operational, cultural, managerial, and communication barriers, developing nations can create a more inclusive, efficient, and sustainable healthcare system. If effectively implemented, the recommendations from this study can enhance NGO-government partnerships, improve healthcare service delivery, and ultimately contribute to better healthcare access and outcomes for vulnerable populations. Future research and policy actions should focus on sustained collaboration efforts, ensuring that NGOs and government agencies work as equal partners in the pursuit of universal health coverage and improved public health outcomes.

CRediT authorship contribution statement

Ali Vafae Najar: Conceptualization. **Esmat Pardasi:** Investigation, Data curation. **Mehrdad Salehi:** Validation, Methodology. **Bahare Boshrouei Shargh:** Writing – review & editing, Validation. **Elaheh Hooshmand:** Writing – original draft, Visualization, Supervision.

Ethics approval and consent to participate

This research was approved by the Medical Ethic Committee of Mashhad University of Medical Sciences (IR.MUMS.REC.1395.352). All participants provided written informed consent form for the various aspects of data collection. It was also implemented in accordance with the principles and regulations of confidentiality and privacy.

Availability of data and material

The datasets used and/or analyzed during the current study are available from the corresponding author on reasonable request.

Transparency statement

All methods and materials used in this research are described in detail in the methods section of this article to allow for replication.

Declaration of the use of AI

The authors acknowledge the use of artificial intelligence tools (ChatGPT by OpenAI) in the preparation of this manuscript, specifically for improving writing clarity, suggesting structure, and language editing. All core content, including study design, data analysis, interpretation of results, and conclusions, were developed solely by the authors. The authors bear full responsibility for the final content of the article.

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Declaration of competing interest

The authors declare that they have no known competing financial interests or personal relationships that could have appeared to influence

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